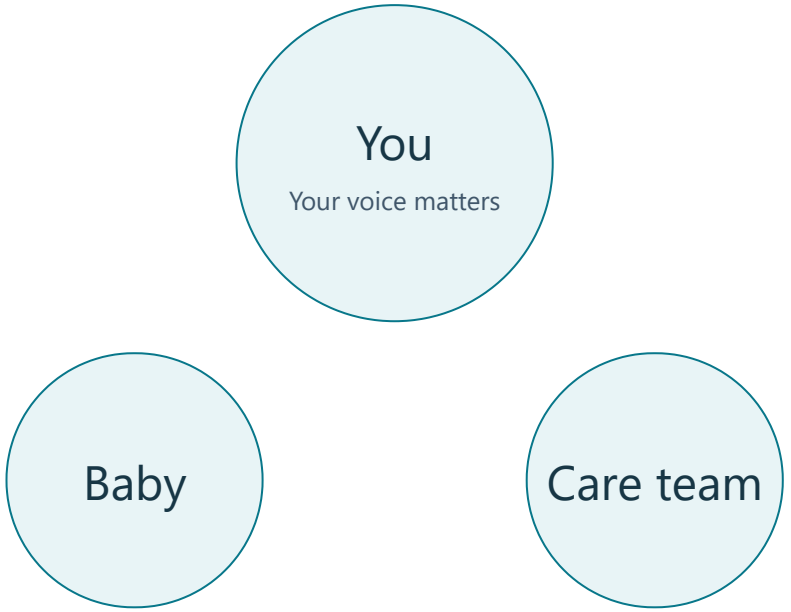
PREGNANCY WITH A HIGHER BMI

Your pregnancy care should fit you.

Your care. Your choices. Your birth plan.

A guide for patients with a BMI above 30, including extra planning for a BMI of 40 or higher.

Dr. Chukwuma Onyeije, MD
Atlanta Perinatal Associates · OpenMFM



> Your health, your baby's health, and your preferences shape the plan.

Sources: 1 2

YOUR ROADMAP

Leave with a plan for visits, birth, and recovery.

01

During pregnancy

Understand your visits and tests.

→

02

Planning birth

Discuss timing, options, and pain relief.

→

03

After birth

Know your follow-up and warning signs.

> Ask: “What is the next step in my care?”

Sources: 1 10

UNDERSTANDING BMI

BMI is one part of your health picture.

BMI, or body mass index, uses height and weight. Your team uses your weight before pregnancy, or an early-pregnancy estimate.

Class I

30–34.9

BMI

Class II

35–39.9

BMI

Class III

40+

BMI

Class III obesity is sometimes called “morbid obesity.” These terms describe a medical category, not your worth or your whole health.

> Your health history matters alongside the number.

Sources: 1 2

WHY FOLLOW-UP MATTERS

Extra care helps your team watch for problems.

Your health

Checks look for high blood sugar and high blood pressure.

Your baby's health

Scans and, when needed, extra testing help assess growth and well-being.

Some risks, including stillbirth, rise with higher BMI. Your team uses your full health picture to plan care.

> An increased risk does not mean a complication will happen.

Sources: 1 2 7

YOUR FIRST VISITS

Your health history helps shape the plan.

Tell us about your health

Blood pressure, diabetes, past blood clots, and any weight-loss surgery.

Tell us about sleep and medicines

Loud snoring, pauses in breathing, and all medicines or supplements, including weight-loss medicines.

> “What in my history changes the care I need?”

Sleep apnea means breathing repeatedly stops and starts during sleep. Tell your team if you use CPAP, a device that helps keep your airway open.

Sources: 2

DAILY HABITS

Focus on nourishment and steady habits.

A usual weight-gain range

11–20 lb

About 5–9 kg total for one baby when BMI before pregnancy is 30 or higher.

Make the plan work for you

Choose nourishing meals and movement you can manage. Ask about a dietitian. If safe for you, build toward 150 minutes of moderate activity each week.

> Your team can adjust the goal for nutrition and baby’s growth. Do not start a weight-loss diet during pregnancy.

Sources: 3 4

DIABETES SCREENING

Diabetes checks happen at different times.

Early pregnancy

Testing based on risk can look for diabetes that was already present before pregnancy.

24–28 weeks

Routine screening checks for gestational diabetes: high blood sugar first found during pregnancy.

A normal early result does not replace the usual 24–28 week check. If diabetes is already diagnosed, follow your diabetes care plan.

Sources: 5

PREVENTING PREECLAMPSIA

Ask whether aspirin belongs in your plan.

Preeclampsia is a pregnancy blood-pressure disorder that can affect organs. Low-dose aspirin helps prevent it in some patients.

01

Review risks

BMI above 30 is one moderate risk factor.

→

02

Look at the full picture

Other risks determine whether aspirin is recommended.

→

03

Agree on a plan

Ask when to start and how to take it, if prescribed.

> Do not start aspirin on your own. Ask your care team early in pregnancy.

Sources: 6

ULTRASOUND

Repeat views do not automatically mean a problem.

01

First scan

Some details may be hard to see.

→

02

More views

A follow-up scan may complete the picture.

→

03

Next steps

Your team explains any finding that needs care.

> Ask: “Was something abnormal, or was the view incomplete?”

Sources: 1 2

EXTRA BABY CHECKS

Extra checks depend on your BMI and health.

These tests check the baby's heartbeat and sometimes fluid or movement. Weekly testing **may be considered**:

BMI **40+** — **34** weeks onward

BMI **35–39.9** — **37** weeks onward

Based on BMI before pregnancy. Other conditions may change the schedule. BMI 30–34.9 alone does not set this extra-testing schedule.

> Testing helps guide care but cannot prevent every problem. Agree on a schedule with your team.

Sources: 7

WHEN TO GIVE BIRTH

Your birth date is a shared decision.

Discuss induction from 39 weeks

Induction means using treatment to start labor. It may be an option, based on your health and preferences.

If pregnancy continues

Agree on follow-up, baby checks, and when to revisit the delivery plan.

BMI alone is not a reason for birth before 39 weeks. A medical complication may require earlier delivery.

Sources: 2 8

HOW TO GIVE BIRTH

A higher BMI does not automatically mean a cesarean.

> “What makes vaginal birth or cesarean the better choice for me?”

Vaginal birth

Can be an option. Your prior births and how labor progresses matter.

Cesarean birth

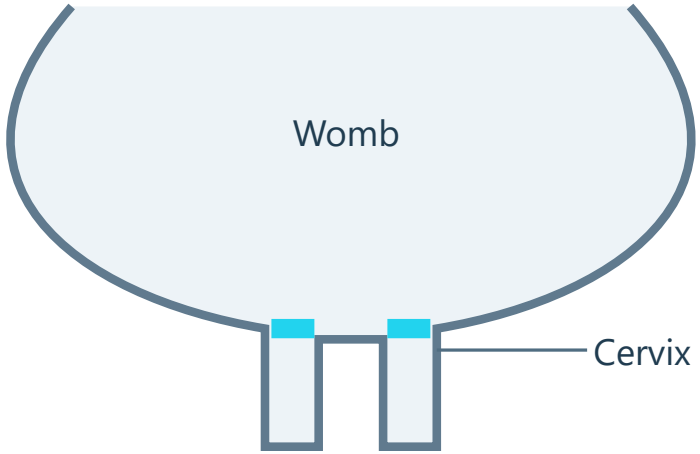
May be needed for a separate medical reason or a problem during labor.

> A prior cesarean needs its own discussion about safe birth options.

Sources: 1 2

UNDERSTANDING INDUCTION

Induction often starts by preparing the cervix.



Simplified drawing; not to scale.

The **cervix** is the opening of the womb.

Ripening means softening and opening it before labor gets going.

01

Prepare

A small balloon, medicine such as misoprostol, or both.

→

02

Reassess

Check the cervix and the baby's heartbeat.

→

03

Support labor

Oxytocin through an IV may help contractions.

Your team adjusts the steps to your needs.

Sources: 8 9

WHAT THE EVIDENCE SAYS

No single induction method is best for everyone.

What we know

A balloon plus medicine can shorten induction in suitable patients overall.

What remains uncertain

Studies have not established one best approach for class III obesity. Larger medicine doses are not automatically better.

Prior cesarean or major womb surgery? Tell your team. Misoprostol is not used for term induction in these situations because it can increase the risk of the womb scar tearing.

Sources: 9 11 12

DURING INDUCTION

A longer induction does not always mean failure.

Induction can take longer with a higher BMI.

01

Check

Contractions, baby's heartbeat, and your well-being.

→

02

Reassess

Is the cervix changing? Is it safe to continue?

→

03

Decide together

Give more time or change the plan when needed.

> Ask: "What are we waiting for, and what would make the plan change?"

Sources: 2 11 12

PAIN RELIEF

Plan pain relief before labor becomes urgent.

Talk before labor

An anesthesia specialist helps plan pain relief and care if surgery is needed.

Share what matters

Past epidural or anesthesia problems, sleep apnea, and CPAP use.

An epidural uses a small tube in your back to give pain-relief medicine. Placement can take extra time, so discuss when to request it.

> Ask about your first choice and a backup plan.

Sources: 2

AFTER BIRTH

Recovery includes a plan to prevent blood clots.

01

Move safely

Ask when to get up and walk after birth.

→

02

Use compression

After cesarean, leg sleeves help blood flow until you are fully walking.

→

03

Review medicine

Some patients also need a blood thinner, based on their full risk profile.

Before discharge, ask what prevention you need at home and for how long.

Sources: 13

WHEN TO GET HELP

Know the symptoms that need help now.

Call 911 for an emergency

Chest pain, severe trouble breathing, fainting, or heavy bleeding with weakness or dizziness.

Get medical care right away

Severe headache, vision changes, one-sided leg pain or swelling, or heavy bleeding. Call maternity care right away if baby moves less than usual.

Do not wait for the next visit. If you cannot reach your team, go for urgent assessment. These warning signs matter during pregnancy and after birth.

Sources: 10

ONGOING CARE

Your care continues after birth.

Before discharge

Know who to call and when your next visit is.

Early follow-up

Review blood pressure, healing, sleep, and emotional health.

Longer-term care

Plan ongoing diabetes care, nutrition support, and future pregnancy goals.

Had gestational diabetes? Ask when and where to complete your postpartum diabetes test.

Sources: 2 5

TAKE-HOME MESSAGES

Remember these five steps.

Keep your visits
01

Nourish yourself
02

Agree on testing
03

Make a birth plan
04

Know when to get help
05

Your BMI helps plan care. It does not tell your whole story.

You deserve clear explanations and respectful care at every step.

Sources: 2 3 5 7 10

QUESTIONS TO ASK

Questions to Ask Your MFM Specialist

MFM means maternal-fetal medicine: specialists in higher-risk pregnancy.

01 Which checks do I need, and why?

02 Should I take low-dose aspirin?

03 When should we plan delivery?

04 Which induction options fit my history?

05 What is my pain-relief plan?

06 What is my recovery and follow-up plan?

Bring this page to a visit. Write down your next step:

Sources: 2 6 7 8 9 13

SOURCES & FURTHER READING

Reliable sources help you keep learning.

01

PATIENT RESOURCE

ACOG. Obesity and Pregnancy. FAQ182.

02

GUIDELINE

ACOG. Obesity in Pregnancy. Practice Bulletin No. 230. Obstet Gynecol. 2021;137(6):e128–e144. doi:10.1097/AOG.0000000000004395.

03

PATIENT RESOURCE

NIDDK. Health Tips for Pregnant Women.

04

PATIENT RESOURCE

ACOG. Exercise During Pregnancy.

05

GUIDELINE UPDATE

ACOG. Screening for Gestational and Pregestational Diabetes in Pregnancy and Postpartum. Obstet Gynecol. 2024;144(1):e20–e23. doi:10.1097/AOG.0000000000005612.

06

PRACTICE ADVISORY

ACOG/SMFM. Low-Dose Aspirin Use for the Prevention of Preeclampsia and Related Morbidity and Mortality. 2021.

07

GUIDELINE

ACOG. Indications for Outpatient Antenatal Fetal Surveillance. Committee Opinion No. 828. Obstet Gynecol. 2021;137:e177–e197.

08

PATIENT RESOURCE

ACOG. Labor Induction.

09

GUIDELINE

ACOG. Cervical Ripening in Pregnancy. Clinical Practice Guideline No. 9. Obstet Gynecol. 2025;146(1):148–160. doi:10.1097/AOG.0000000000005951.

10

PATIENT SAFETY

CDC. Urgent Maternal Warning Signs and Symptoms. HEAR HER Campaign.

11

STUDY: SECONDARY ANALYSIS

Croll DMR, De Vaan MDT, Moes SL, et al. Methods of induction of labor in women with obesity: A secondary analysis of two multicenter randomized controlled trials. Acta Obstet Gynecol Scand. 2024;103:470–478. doi:10.1111/aogs.14737.

12

STUDY: RANDOMIZED TRIAL

Saucedo AM, Alvarez M, Macones GA, Cahill AG, Harper LM. Optimal misoprostol dosing among patients with a body mass index greater than 30: a randomized controlled trial. Am J Obstet Gynecol. 2024;230:565.e1–565.e16. doi:10.1016/j.ajog.2024.02.004.

13

GUIDELINE

SMFM. Consult Series #51: Thromboembolism prophylaxis for cesarean delivery. 2020. Reaffirmed 2025.

Sources checked November 4, 2026. Studies inform care; they do not predict your individual outcome. Source links open in a new tab.

YOUR NEXT STEP

Use this guide to start a conversation.

Bring your questions. Ask for a plan you understand.

Educational information

This guide supports conversations with your care team. It does not replace personal medical advice or urgent assessment. It focuses on US care for pregnancy with one baby; other situations need different plans.

Privacy

This presentation asks for no personal information and contains no forms, tracking scripts, or stored patient data. External source websites have their own privacy policies.

OpenMFM Presentation Standard v1.0 · Prepared November 4, 2026

> Your care should reflect your health, your baby, and what matters to you.